



POLICY

Risk Assessment and Management

Scope (Staff):	All staff
Scope (Area):	CAMHS

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Only use this box if relevant (e.g. clinical document or available on an external website)

Contents

Aim	2
Risk.....	2
Background.....	3
Definitions	3
Principles	6
Documentation Requirements for Risk Assessment.....	10
Developing a Risk Management Plan	11
Communication.....	14
Reviews of risk.....	14
Escalation of Risk	15
Training.....	16
Compliance.....	16

Aim

To establish the requirements for risk assessment and management by all Child and Adolescent Mental Health Services (CAMHS) staff.

The aims of this policy are:

- To promote high standards of risk assessment and management within CAMHS
- To guide CAMHS staff in the practice of risk identification to improve both safety and quality of service delivery
- To outline the process of developing risk management plans after risk identification and assessment has occurred
- To support CAMHS staff with risk reviewal, risk monitoring and risk escalation

Risk

Risk refers to ‘the likelihood of an event occurring, which may have harmful outcomes for the person or others¹’. Failure to identify, assess and subsequently manage clinically significant risk may consequent in further adverse emotional, mental and physical health outcomes to self and others, and also a compromise of staff safety.

Risk to self²:

- Self-neglect, self-harm and repetitive self-injury
- Suicide and attempted suicide
- Misadventure (missing or absent without leave)
- Physical deterioration including drug and alcohol misuse, and other medical conditions (including medical conditions secondary to eating disorders)
- Quality of life including dignity, reputation, social and financial status

Risk to others²:

- Harassment
- Stalking or predatory intent
- Violence and aggression
- Property damage
- Public nuisance and reckless behaviour that endangers others

Risk from others (especially considering vulnerable persons)²:

- Physical, sexual or emotional harm or abuse by others
- Social or financial abuse or neglect by others

Background

The Child and Adolescent Mental Health Service is committed to supporting staff to deliver safe care through routine clinician training and adherence to standardised strategies for recognising and responding to children's risk to themselves and others¹. This document should be read in conjunction with the following documents; [Mental Health Act WA 2014](#), [Clinician's Practice Guide to the MHA 2014](#), [Chief Psychiatrist WA Standards of Clinical Care 2015](#); [Clinical Care of People Who May be Suicidal state-wide policy](#), [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#), [CAMHS Sexual Safety policy](#), [CAHS Child Safeguarding and Protection Policy](#) and [CAHS Mandatory Reporting of Child Sexual Abuse policy](#). It is recognized that there will be situations where exceptions to this policy will be necessary, to act in the best interests of patient care. These exceptions must be justified and documented within the medical record.

Definitions

Child: An infant, child, adolescent, or young person under the age of 18 years.

Child Abuse: All forms of physical and/or emotional ill-treatment, sexual abuse, neglect or negligent treatment or commercial or other exploitation, resulting in actual or potential harm to the child's health, survival, development or dignity in the context of a relationship of responsibility, trust or power³.

Child Sexual Abuse: In relation to a child includes sexual behaviour in circumstances where a child is exposed to, or involved in, sexual activity that is inappropriate to the child's age and developmental level. It includes circumstances where⁴:

- The child is the subject of bribery, coercion, a threat, exploitation or violence;
- The child has less power than another person involved in the behaviour;
- There is a significant disparity in the developmental function or maturity of the child and another person involved in the behaviour

Some examples of child sexual abuse include (but are not limited to):

- Letting a child watch or read pornography;
- Allowing a child to watch sexual acts;
- Fondling the child's genitals;
- Having oral sex with a child;
- Vaginal or anal penetration;
- Using the internet to find a child for sexual exploitation;

Family: Any parent, legal guardian (including those under the Department of Communities, Child Protection and Family Support), or identified carer of the child, a close family member or a nominated person (MHA 2014, s.7). For a child of Aboriginal or Torres Strait Islander descent, this may be a person from their kinship group who is recognised under customary laws and traditions as their carer. For a legal definition, refer to the [Mental Health Act 2014](#), s. 281 – Close Family Members.

Indecent Assault: Refers to a person who unlawfully and indecently assaults another person⁷. The term indecent may refer to behaviour of a sexual nature, that is considered to be contrary to community standards of decency.

Key Stakeholders: Professionals involved in the care of the child or support of the family, including but not limited to referrer, referring agency, school, group home, Department of Communities, Child Protection and Family Support, GP.

Non-suicidal Self-Injury (NSSI): Any deliberate, self-inflicted bodily injury where there is no evident intention to die. The absence of suicidal intent is either reported by the patient or can be inferred by frequent use of methods that the patient knows, by experience, not to have lethal potential. Non-suicidal self-injury/harm represents a maladaptive coping mechanism to regulate overwhelming emotions and to endure life. This includes but is not limited to self-poisoning, overdose, cutting etc⁵.

Positive Risk Management:

Protective Factors: Factors such as internal resilience factors (such as coping), community reintegration factors (such as work or leisure activities), treatment related factors (such as motivation for change or medication), social factors (such as peer support), or environmental factors (such as supervised housing)⁶.

Risk Assessment: A gathering of information and an analysis of the potential outcomes of identified behaviours. It includes identifying specific factors of relevance to a consumer and their family or carers, and the context in which they may occur. This process requires linking historical information to current circumstance, to anticipate possible future change¹.

Risk Factor: Particular features of illness, behaviour or circumstances that alone or in combination lead to an increase risk of an adverse event or outcome occurring¹.

Risk Management: Actions which aim to minimise the likelihood of adverse events within the context of overall management of a consumer. It provides the opportunity for targeted intervention to minimise the causative factors to achieve the best outcome and deliver safe, appropriate, and effective care. Risk management can be both at an individual and systemic level¹.

Sexual Activity: A person engages in sexual activity if the person allows a sexual act to be done to the person's body, does a sexual act to the person's own body or the body of another person, or otherwise engages in an act of an indecent nature⁷. Note that sexual activity may be consensual. If sexual activity is non-consensual, this is considered to be sexual assault or indecent assault. Additionally, sexual activity with a child (under 16) is considered to be a sexual offense, as is sexual activity with a child over age 16 when they are under the care, supervision or authority of the offender.

Sexual Assault: Sexual assault describes a broad range of non-consensual sexual activity and non-consensual sexual behaviours that are committed against a person. These crimes include (but not limited to) sexual penetration without consent and indecent assault. For further information refer to the [WA Criminal Code 2013](#).

Sexual Harassment: Unwelcome conduct (below) of a sexual nature which makes a person feel offended, humiliated and/or intimidated where that reaction is reasonable in the circumstances⁸. It is not sexual interaction, flirtation, attraction or friendship which is invited, mutual, consensual or reciprocated⁹. Examples of sexual harassment:

- Unwelcome touching, hugging or kissing, staring or leering;
- Suggestive or offensive comments/jokes; insults/taunts of a sexual nature;
- Unwanted invitations to go out on dates or requests for sex;
- Intrusive questions about an individual's private life or body;
- Unnecessary familiarity, such as deliberately brushing up against someone;
- Insults or taunts of a sexual nature, sending sexually explicit emails/texts;
- Behaviour that is an offence under the criminal law (physical assault, indecent exposure, sexual assault, stalking, obscene communications)

Sexual Safety: Refers to being and feeling psychologically and physically safe, including being free of, and feeling safe from, behaviour of a sexual nature that is unwanted or makes another person feel uncomfortable, afraid or unsafe. This includes sexual offences, indecent assault, sexual assault and harassment. It also extends to being spoken to using sexualised language or observing other people behaving in a sexual disinhibited manner, including nakedness and exposure or masturbation, being made to watch or shown pornographic images and lacking privacy and dignity when naked¹⁰. Incidents that compromise sexual safety include the types of behaviour that can breach and/or compromise the sexual safety of a child and include:

- Sexual assault or harassment (by fellow patients, visitors or staff);
- Consensual sexual activity in an inappropriate context or setting (such as an inpatient or day service);

- Sexually disinhibited behaviour

Sexually Disinhibited Behaviour: Difficulty controlling sexual behaviour where sexual thoughts, impulses or needs are expressed in a direct or disinhibited way such as in inappropriate situations, at the wrong time or with the wrong person. This behaviour arises for a variety of reasons that can include a mental illness or disorder, cognitive impairment, alcohol intoxication, illicit drug use or the side effects of medication¹¹.

Structured Clinical Judgement: Judgement about risk based on a combination of the evidence base for risk factors, individual patient assessment including the child and family's view of their own experience and circumstances, together with clinical experience and knowledge of the consumer¹².

Suicidality: Thoughts and behaviours related to suicide, including suicidal ideation, suicidal intent, suicide planning and suicide attempt.

Principles

The following key principles guide risk assessment and management in CAMHS.

- Risk cannot be eliminated, only minimised, therefore clinicians must consider that risk is fluid and can change over very short timeframes. While some risk factors are long-term, stable and indicate an individual's general propensity to harm self or others, other factors are short term, dynamic and capture the fluctuating nature of risk¹²
- Risk assessment is a dynamic process that must identify the drivers and antecedents of risk and what may lead to fluctuations of risk. This assessment must inform the development of a management plan to mitigate risk;
- Risk assessment and management must be informed by consultation with key stakeholders with the consent of children and their families and input from the multidisciplinary team;
- Clinical supervision and training is available to support staff in undertaking risk assessment and management planning;
- Children and their families must be actively involved in identifying risk factors, with the emphasis of care on transparently and collaboratively developing the risk management plan to empower and build on the child's strengths^{12,13};
- Positive risk management recognises all decisions carry some element of risk, and this approach to developing the risk management plan builds on the consumer's strengths and enhances their recovery, is based on a trusting therapeutic relationship and uses least restrictive practice^{12, 13};
- Risk management must be informed by recovery-oriented practice, be person-centred, trauma-informed, and developmentally appropriate and provided by staff who are culturally competent^{12, 13};

- Risk management must take into account cultural and social diversity, including considerations around Aboriginality, gender, ethnicity, pregnancy or maternity, disability, religion, spirituality and/or belief, neurodiversity, sexual diversity and gender identity, socioeconomic status, medical condition and age;
- If the child is Aboriginal, significant others from the child's community may also be involved in identifying risk factors and developing the risk management plan;
- Appropriate assistance and reasonable adjustments must be provided during the risk assessment and management process for children and their families who are from Culturally and Linguistically Diverse (CaLD) backgrounds e.g. those not speaking, reading or writing English, those with communication difficulties such as hearing, oral, or learning impairments;
- Upon occurrence of sentinel events or adverse outcomes, they are reviewed as opportunities for service improvement, supported by a 'no-blame' culture¹;

Risk Assessment

A formal, clinical risk assessment should be completed to evaluate patient risk of harm to self and to others, noting the risk assessment may have been completed via phone with a parent / carer. It involves the clinician making a structured, clinical judgement from an evidence base of risk factors; the individual patient assessment (including the child and family's view of their own experiences and circumstances), in addition to the clinician's own clinical experience and knowledge of the child and family^{12,13}.

Assessment of risk occurs:

- As part of the initial mental health assessment (incl. Triage (initial contact) CHOICE appointments)
- Where there is significant change in mental state, circumstances or life events
- At the request of the child, the child's family, or key stakeholders
- On discharge from, referral to, or transfer within and between clinical settings
- When there is a significant change in the intensity of care being provided
- At times of risk reviews (Triage / CRAMP completion)

For further information on when to conduct a risk assessment, refer to the following documents:

- [CAMHS Admission to Inpatient Service Procedure](#)
- [CAMHS Clinical Assessment Policy](#)
- [CAMHS Clinical Handover](#)

- [CAMHS Progressive Risk Monitoring](#)
- [PCH CAMHS Inpatient Leave](#)
- [CAMHS Sexual Safety](#)
- [CAMHS Managing Clinical Risk After Disclosure of Child Sexual Abuse](#)
- [PCH CAMHS Transfer and Care of Sedated Patients to Ward 5A](#)
- [Increased Clinical Observation Levels in CAMHS Ward 5A](#)

Assessment of risk must encompass:

- A detailed evaluation of all aspects of risk;
- A comprehensive diagnostic assessment and formulation - note that in cases of high risk, there should be consultation with a senior clinician and/or psychiatrist;
- A thorough determination of the psychosocial circumstances contributing to the clinical presentation, which involves assessment of the family's ability to safeguard their child and contain risk¹¹;
- If the child is Aboriginal, to the extent that is practicable and clinically appropriate, the assessment must be conducted with an Aboriginal Mental Health Worker (AMHW) *and* representation of a significant member/s of the child's community, including elders and traditional healers (MHA, s.50)

For children who are patients within the CAMHS Inpatient Unit (Ward 5A):	An initial risk assessment must be completed within 2 hours of admission.
For children who have demonstrated suicidality:	A psychiatric diagnostic assessment and formulation must be completed.
For children requiring treatment for mental health conditions, who may be at risk of becoming violent or aggressive:	Assessment of risk must include (but is not limited to) the following considerations: • Serious mental illness, including neurodevelopmental comorbidities • Substance abuse • Childhood experience • Self-harm • Systemic issues, including any child protection concerns in the current context as per the Principles

	and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour ⁵ When the risk to others is assessed as high, sustained over a period of time, or associated with potential offending (arson, all sexual activity and sexual behaviour, sexually inappropriate behaviour, threatening or stalking behaviours), consideration should be given to seeking further opinion. This opinion should be firstly from the Consultant Child and Adolescent Psychiatrist within the team but may later include further specialised clinicians (internal or external to CAHS). Importantly, risk should be acknowledged, documented and communicated within the Team, and not held by individual clinicians^{4,5}
For children requiring treatment for mental health conditions, who may be at risk of participating in sexual activity or exhibiting sexual behaviours:	Assessment of sexual vulnerability, sexual risk and sexual safety must occur in accordance with the the CAMHS Clinical Assessment policy, CAMHS Sexual Safety Guidelines and the SSCD Initial Assessment. It includes (but is not limited to) the following considerations: - Recognize priority child and adolescent groups that are particularly vulnerable to sexual or predatory risk - Recognizing behaviours of other children, adolescents and adults who may pose a sexual risk to others - Assessment of patients who may pose as a sexual risk to others and/or compromise the sexual safety of others
For children who are needing to be transported from Ward 5A to an alternate location for further treatment:	When a child needs to be transported to an alternate location for further examination or admission, an assessment for risk of harm to self or others during transportation must be completed. This assessment will be used to determine the least restrictive means of transport that is reasonably available to safely transport the child, including transport under the MHA. See the Mental Health Patient Transport Risk Rating form and the resource section for links to the MHA transport forms. Risk management during transportation must also comply with relevant Commonwealth, State and CAHS transport policies and

	guidelines; and with Requesting Transport Officers and WA Police Assistance in Transporting Mental Health Patients.
--	---

Documentation Requirements for Risk Assessment

If risk is identified following completion of the initial assessment, a Statewide Standardised Clinical Documentation (SSCD) Risk Assessment and Management Plan (RAMP) must be completed. It is available in both [printable](#) and [writeable](#) formats, and all sections must be completed.

The following should be documented in the patient's medical record:

- If no risk is identified at the initial assessment
- Evidence of any re-assessment
- If it is not possible to complete a RAMP, reasons for this must be documented

Completion of scores regarding the type of risk on the CAMHS SSCD RAMP:

- Record one whole number score per type of risk only (that is 0, 1, 2, 3, or 4).
- Use the higher score if there is uncertainty

When a score of 3 or greater* is recorded in one or more categories on the CAMHS SSCD RAMP, clinicians must:

- Step 1: put an alert on PSOLIS
- Step 2: consult with a psychiatrist or a senior clinician regarding the assessment and development of risk mitigation strategies in the management plan
- Document completion of the above steps in the management plan (i.e. page 3 of RAMP) and the PSOLIS crisis management plan
- Additionally, for children in the inpatient unit, ward 5A:
 - Clinical alerts must be reviewed or added to PSOLIS by the clinician:
 - After all clinical reviews
 - Before leave
 - Before discharge

***Note:** some cases may necessitate such actions even if a score of 3 or greater is **not** recorded (e.g. client history of increased risk). Where the child or family lack the decisional capacity to assess the implications of their actions, **clinicians have a clear duty of care to intervene in the best interest of the child** to support safety of the child and others. When a mutual agreement to the safety plan cannot be reached with children and their families, their views are to be acknowledged, decisions discussed with them and continued efforts made to assist the individual regain control over their

own safety, this may include recommendation of treatment in a more restrictive setting and/or consideration of treatment under the Mental Health Act. The differing views and reasons for decisions should be documented in the child's medical record. All violent and aggressive incidents must be reported via Datix CIMS^{11,12}.

For incidents relating to sexual safety, any allegation of sexual contact in an inpatient setting or allegation of sexual assault is a serious incident and must be reported through the mental health service clinical reporting system (Datix CIMS) as soon as practicable (within 48 hours) and investigated, as detailed in the [Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#) and the [WA Health Clinical Incident Management Policy](#). This should include involvement of the Child Protection Unit to ensure independence of investigation. Note that staff must also inform their immediate line manager and the consultant psychiatrist in the event where sexual safety is compromised.

Developing a Risk Management Plan

Risk must be assessed accurately to ensure appropriate risk mitigation strategies as part of the broader risk management plan. In cases where children and their families are not able to be involved in the risk assessment and management process, reasons for this must be clearly documented in the child's medical record.

Where risk is identified and assessed, clinicians must complete a comprehensive risk management plan collaboration with the child, the child's family and the appropriate multi-disciplinary team^{4,5}. The following should be considered:

- Types of risk and to whom
- Recognizing that risk is unique to the individual, by identifying specific triggers and circumstances which may compromise patient safety
- Formulation of strategies to reduce risk and enhance safety⁴, which should;
 - Take into account the views of the child and their family in identifying which interventions are likely to work
 - Link strategies to the child's strengths, life goals and overall recovery and enhance the child's capacity to keep themselves safe
 - Empower the family to safeguard the child by being active participants in the risk management plan
 - Identify the actions to be taken (including when and by whom) in the event of a crisis
 - Include protective factors
- Identify how the child, their family, and the clinician will regularly monitor the child's safety

- Description of how responsibility for safety will be shared by specifying agreed actions for the child, their family, and clinicians in implementing the safety plan
- Rationale for decisions made e.g. hospitalization versus outpatient care
- Specify review dates
- Detail the responsibilities of clinicians for follow up, including presentation at the multi-disciplinary team clinical review
- Additionally for CAMHS inpatients in Ward 5A, on admission a plan must be developed which includes communication of goals, risks and management, access and allocation of ward areas and identification of clinical observation levels (including use of bathrooms), all to be documented on page 3 of the Statewide Standard Clinical Document Risk Assessment and Management Plan (SSCD RAMP)
- With regards to safety plans, this format may vary between teams. The safety plan can be incorporated as part of the RAMP or other individualised recovery informed plans however at a minimum, the management plan on PSOLIS and page 3 of the RAMP must be completed. Children and families (at a minimum) must be provided with a copy of page 3 of the RAMP^{11,12}

For children requiring treatment for mental health conditions, who may be at risk of becoming violent or aggressive:

In addition to the above the safety plan must also include:

- Formulation of strategies that explore distractions that the consumer has identified to reduce the likelihood of violence or aggression
- Identification of the actions to be taken, when and by whom in the event of a crisis
- Childhood experience
- Self-harm
- Systemic issues, including any child protection concerns in the current context as per the [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)⁵
- When the risk to others is assessed as high, sustained over a period of time, or associated with potential offending (arson, sexually inappropriate behaviour, threatening or stalking behaviours), consideration should be given to seeking further opinion. This opinion should be firstly from the Consultant Child and Adolescent Psychiatrist within the team but may later include further specialised clinicians (internal or external to CAHS). Importantly, risk should be acknowledged, documented and communicated within the Team, and not held by individual clinicians^{4,5}

- Note also that the care delivery system may contribute to the child's frustration and hence the likelihood of aggression or violence. Teams should consider service configuration and provision to minimise this risk and maximise safety for all concerned.
 - Therapeutic Crisis Intervention (TCI) principles, including setting conditions should be considered by the CAMHS Inpatient Unit Ward 5A, community based teams, day and outpatient programs
 - For the CAMHS Inpatient Unit Ward 5A, refer to the [Ward Milieu and Patient Cohort Management work instruction](#) and the [Safewards model](#)
 - For community based teams, day and outpatient programs, refer to the [CAMHS Home and Community Visits procedure](#), the [WorkSafe - aggression webpage](#), or the [CAHS Work Health Safety and Wellbeing](#)

For children requiring treatment for mental health conditions, who may be at risk of participating in sexual activity or exhibiting sexual behaviours:

Children identified as vulnerable to, or at risk of sexual activity, assault or harassment should have a detailed treatment or care plan that:

- Recognizes the characteristics of children and adolescents who are vulnerable to sexual or predatory risk (refer to the [Chief Psychiatrist's Sexual Safety Guidelines 2020](#) and the [CAMHS Sexual Safety Guidelines](#))
- Recognizes the implicit behaviours of those who may pose a sexual risk to others (refer to the [Chief Psychiatrist's Sexual Safety Guidelines 2020](#) and the [CAMHS Sexual Safety Guidelines](#))
- Consider medical records or past medical history of patients that outline behaviours that may cause them to pose a sexual risk to others
- Is identified in the management plan and documented in the medical record;
- Is known to all staff (not just the treating doctor or primary nurse);
- Specifies the risk behaviours and/or vulnerabilities;
- Details the approach to be adopted to protect the child;
- Describes specific interventions; and
- Is regularly reviewed by the treating team at inpatient review meetings or multidisciplinary team review meetings in community settings and at handover.

Where possible the plan should be developed in collaboration with the child and their families and carers. The child should be advised, to the extent their mental status allows, what staff are doing to keep them safe and how they might contribute to their own safety, regardless of the risk level identified. The risk to the child's

reputation and potential consequences of their actions should also be considered and discussed.

Communication

In situations where the child does not consent to information about their risk being communicated to families or other key stakeholders, it must be discussed with the child about what can be communicated to others and the form in which it is communicated^{4,5}. This should include conforming with the child's wishes as much as possible, unless duty of care overrides confidentiality when risk has been identified. This discussion and outcome (i.e. the information that is communicated and to whom) must also be documented in the medical record. See [CAMHS Consent Policy](#) for information around the use of the CAMHS Consent to Exchange of Information Form (MR510.07).

Reviews of risk

Risk assessment, safety plans (including crisis management plans) and PSOLIS alerts must be reviewed on a routine, ongoing basis, in collaboration with the MDT^{11,12}. The frequency of reviews should be informed by a range of factors, including outcomes of the scheduled reviews and the level of risk. As a minimum, PSOLIS alerts and Crisis Management Plans must be reviewed every 3 months, in collaboration with the MDT. For further information regarding reviews of risk, see relevant MDT Clinical Reviews Policies and the [PSOLIS Alerts Process](#).

Clinicians, in collaboration with child, family and the MDT, must review risk and update the safety plan:

- At the request of child or family or key stakeholders;
- When there is significant change in mental state, circumstances, or life events;
- Where it is identified that safety arrangements need to be reviewed;
- At times that are recognised to be periods of heightened risk;
- And at points of significant transitions in care including:
 - On discharge, referral or transfer between clinical settings; and
 - When there is a significant change in intensity of care being provided (e.g. cessation of nursing special).

Where the child is active with CAMHS and have been discharged from an inpatient unit risk reviews must also be completed:

- following self-discharge; and
- in the first week after the discharge period.

For children requiring treatment for mental health conditions, who may be at risk of becoming violent or aggressive:

In addition to the above, the safety plan must be updated as part of any post-incident review, where it is identified that safety arrangements may need to be reviewed and at times that are recognised to be periods of heightened risk.¹²

For children who are inpatients within the CAMHS Inpatient Unit (IPU) and Ward 5A:

The CAMHS IPU use the *Progressive Risk Assessment form (MR508)*, which captures risk at a particular time and supports staff to escalate a child's care if deterioration in mental state/increase in risk is identified. For more information, see the [Progressive risk monitoring work instruction](#), which outlines requirements for monitoring of risk three times a day at shift handovers.

In addition to the reviews of risk as above for all children, reviews of risk in the inpatient unit and Ward 5A must also be completed:

- On admission,
- Before consideration of leave,
- On return from being absent without leave from an inpatient setting,
- Post serious clinical incident,
- During the immediate pre-discharge period,
- If indicated by a progressive risk assessment

The [Ward 5A Model of Care](#) details required follow-up after discharge from the CAMHS inpatient unit, which aligns with the potential in increased risk of suicide in the first week after discharge.

For children requiring treatment for mental health conditions, who may be at risk of compromised sexual safety or children displaying sexually inappropriate behaviours:

Risk review should be conducted in line with the [CAMHS Clinical Assessment Policy](#), [CAMHS Sexual Safety Guidelines](#) and Statewide Standardised Clinical Documentation (page 3, Initial Assessment).

Escalation of Risk

The case manager or allocated clinician is responsible for the ongoing assessment and management of each child. These staff members must alert their line manager of any identified risk and must also discuss the risk at the next multidisciplinary team

meeting. The outcome of the multidisciplinary team discussion must be documented in the medical record and incorporated in discussion of plan with the child and family, the management plan and safety plan where relevant.

Reviews of shared care agreements must occur regularly, including when a significant change in the child's circumstances is identified which might impact upon risk.¹

Changes in the child's level of risk must be communicated in a timely way to appropriate stakeholders, including other health professionals sharing clinical responsibility with CAMHS for the mental health care of the child – see the [Shared Care in CAMHS Guidelines](#) for further guidance.

With regards to escalation of sexual safety incidents, consultation should take place in both incidences of re-assessing risk pertaining to sexual safety, and also reporting when sexual safety risk has escalated. Note that in the Ward 5A Inpatient Unit, the care coordinator is responsible for ongoing assessment and management. The shift coordinator and/or consultant psychiatrist must also be alerted to any identified risk.

Training

Directors and Heads of Departments: In conjunction with the CAMHS Learning & Development Coordinator, must ensure staff receive education, training and information on clinical risk assessment and management.

Service Managers and Heads of Service: Must provide orientation and education to CAMHS staff to ensure they understand the clinical risk assessment and management process.

To ensure competency in suicide safety assessment and management, clinical staff must complete both skills training (as part of the CAMHS orientation) and TCI/WAVE training. Non-clinical staff must complete the CAMHS admin de-escalation workshop.

Compliance

- Compliance with this policy will be audited annually with an approved auditing tool
- Auditing of compliance must incorporate the Chief Psychiatrist's *Standard 5: Risk Assessment and Management*¹
- An annual review of all suicide/attempted suicide incidents to identify common factors or trends must be undertaken¹¹
- An annual review of all violent and aggressive incidents for each twelve-month period to identify common factors or trends¹²
- Regular review of service configuration and provision to determine contribution to the risk of children becoming violent or aggressive¹²

- Report is to be tabled annually for discussion at CAMHS Systems and Practices Committee

Related CAHS/CAMHS internal policies, procedures and guidelines
Admission to CAMHS Inpatient Service
CAHS Child Safeguarding and Protection
CAHS Clinical Incident Management
CAHS Mandatory Reporting of Child Sexual Abuse
CAMHS Clinical Assessment Policy
CAMHS Clinical Handover
CAMHS Consent Policy
PCH CAMHS Inpatient Leave
CAMHS Managing Clinical Risk After Disclosure of Child Sexual Abuse
CAMHS Progressive Risk Monitoring
CAMHS Sexual Safety
CAMHS Shared Care Guidelines
CAMHS Transitions in Care Procedure
PCH Transfer and Care of Sedated Patients to CAMHS Ward 5A
PCH CAMHS Increased Clinical Observation Levels in CAMHS Ward 5A
Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour

References and related external legislation, policies, and guidelines	
1.	Chief Psychiatrist WA Standards of Clinical Care 2015
2.	Clinicians' Practice Guide to the Mental Health Act 2014
3.	World Health Organization – Child Maltreatment
4.	Children and Community Services WA Act 2004
5.	Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist
6.	de Vries Robbé M & Willis G (2017). Assessment of protective factors in clinical practice. <i>Aggression and Violent Behavior</i> . 32: 55-63.
7.	Parliamentary Counsel's Office of Western Australia, <i>Criminal Code Act 2013</i> . State Law Publishers: Perth
8.	Sex Discrimination Act 1984
9.	Australian Human Rights Commission. <i>Sexual harassment</i> (2019)
10.	Victoria Mental Health Complaints Commissioner (2018). <i>The Right to be Safe - Ensuring sexual safety in acute mental health inpatients units: Sexual Safety project report</i> . State of Victoria.
11.	NSW Ministry of Health, <i>Sexual Safety of Mental Health Consumers Guidelines</i> , Mental Health, Drug and Alcohol Office (2013). NSW Government: North Sydney NSW.
12.	Best Practice for the Care of People Who May Be Suicidal
13.	Principles and Best Practice for the Care of People with Mental Health Problems Who May Be at Risk of becoming violent or aggressive state-wide policy

This document can be made available in alternative formats on request.

File Path:	CD/25/36867		
Document Owner:	CAMHS Medical Co-Director		
Reviewer / Team:	Policy & Procedure Steering Group, CAMHS Management Committee, CAMHS Medical Advisory Committee, SSAMHS, Ward 5A Nursing Staff, Nursing Coordinator, Aute Care Response Team (ACRT)		
Date First Issued:	August 2013	Last Reviewed:	17.04.2022
Amendment Dates:	April 2019, October 2020, March 2021, June 2022, December 2024	Next Review Date:	*24/05/2026
Approved by:	CAMHS Medical Co-Director	Date:	10.06.2022
Endorsed by:	Safety & Quality, Audit, Risk & Compliance Executive Committee	Date:	08/01/2025
Aboriginal Impact Statement and Declaration (ISD)		Date ISD approved:	
Standards Applicable:	NSQHS Standards:  Child Safe Standards: 1, 2, 3, 4, 5, 7, 10		
Printed or personally saved electronic copies of this document are considered uncontrolled			
 Healthy kids, healthy communities CompassionExcellenceCollaborationAccountabilityEquityRespect Neonatology Community Health Mental Health Perth Children's Hospital			

*Note: CAMHS Director Clinical Services approved an extension of the review date to 24/05/2026.